

CHILDREN'S APPERCEPTION TEST — HUMAN VERSION (CAT-H)

PSYCHOLOGICAL TEST REPORT

SOCIO-DEMOGRAPHIC DETAILS

Field	Details
Name	AP
Age	16 years
Sex	Male
Test Administered	Children's Apperception Test — Human Version (CAT-H; Bellak & Bellak, 1965)
Cards Administered	All 10 cards
Language of Administration	Hindi
Date of Assessment	---
Examiner	---

TEST ADMINISTERED AND RATIONALE

Children's Apperception Test — Human Version (CAT-H):

The CAT-H was administered to explore the patient's underlying personality dynamics, emotional conflicts, defense mechanisms, interpersonal perceptions, and object relations. The CAT-H utilises human figures substituted for the original animal figures of the CAT-A (Bellak & Bellak, 1965) and is designed for children and adolescents, particularly those aged 7–10+ years and those with higher intellectual functioning who respond more productively to human stimuli. The test provides insight into how the patient perceives familial relationships, handles internal conflicts, views parental and sibling figures, and manages anxiety and aggression.

The scoring and interpretation follow Bellak's 10 Variables System, supplemented by Murray's Need-Press Framework for drive analysis.

RESULT AND INTERPRETATION

CHILDREN'S APPERCEPTION TEST — HUMAN VERSION (CAT-H)

The analysis of the patient's CAT-H responses indicates that he was able to understand the task and construct coherent, reality-based narratives with clear temporal sequencing across all 10 cards. The stories were elaborate, detailed, and emotionally invested, reflecting above-average verbal productivity and adequate comprehension of the task demands. However, the content of the narratives reveals significant psychological disturbance characterised by pervasive themes of violence, parental inadequacy, emotional deprivation, and profound feelings of worthlessness.

Main Heroes and Self-Image:

Across the narratives, the heroes varied between parental figures (Cards 1, 7) and child figures (Cards 2, 3, 5, 6, 8, 9, 10), with the patient most consistently identifying with child characters who are emotionally deprived, unrecognised, or in danger. The heroes are predominantly portrayed as burdened, unvalued, and struggling within an unsafe family environment. In Card 2, the hero is explicitly described as someone whose death would cause the "least loss" — a profoundly concerning self-representation. The adequacy of the hero varies significantly across cards: ranging from complete inadequacy and helplessness (Cards 1, 2, 7, 9) to high adequacy through perseverance and self-reliance (Card 8). This variability suggests that the patient possesses latent ego resources that are activated under specific narrative conditions (i.e., when the story framework permits autonomous action), but these resources are overwhelmed by depressive and aggressive content in the majority of narratives. The body image conveyed is one of vulnerability — the self as small, powerless, and at risk of being consumed or destroyed by authority figures.

Main Needs and Drives (Murray's System):

A prominent pattern observed across the stories is the pervasive presence of unmet affiliative needs (nAffiliation) and succorant needs (nSuccorance), particularly directed toward parental figures. The patient demonstrates an intense, unfulfilled desire for emotional connection, recognition, and unconditional acceptance from parents. This is most powerfully expressed in Cards 2 and 6, where the patient portrays himself as the capable but unrecognised middle child whose heroism and competence fail to elicit parental love or attention, in contrast to a favoured younger sibling who receives unconditional affection despite irresponsible behaviour.

Aggressive needs (nAggression) are prominent but are almost exclusively projected onto parental figures rather than expressed by the hero. In Cards 1 and 7, extreme oral-aggressive and cannibalistic content is attributed to father and mother figures respectively, suggesting massive projection of aggressive impulses onto authority figures. The repetition of cannibalistic themes across two cards is clinically significant, indicating oral-aggressive fixation and/or deep incorporation anxieties.

Need for autonomy (nAutonomy) appears frequently but is predominantly forced rather than chosen — the patient's heroes achieve independence not through healthy separation-individuation but through necessity born of parental failure or danger. Card 8 represents the exception, where autonomy is actively pursued through hard work and confrontation.

Need for achievement (nAchievement) is present but consistently undermined by lack of recognition. The patient's heroes are competent and capable but their achievements are systematically ignored or devalued by parental figures.

Need for harm avoidance (nHarm Avoidance) is the most pervasive drive across the protocol, reflecting a fundamental preoccupation with survival in dangerous familial environments.

Conception of the Environment (World):

The conception of the environment is predominantly dangerous, exploitative, and emotionally depriving. Across the majority of narratives, the family home is perceived as a site of violence, neglect, or emotional abandonment rather than safety and nurturance. Authority figures — particularly parents — are consistently portrayed as either actively dangerous (Cards 1, 7: murderous, cannibalistic), emotionally unavailable (Cards 3, 9: busy, absent fathers), blindly ineffective (Cards 6, 8: parents who cannot see or address problems), or conditionally loving (Cards 2, 6: favouritistic parents).

The external world is perceived as offering only institutional rescue (police in Cards 1, 7, 10) or self-generated escape (Card 8). In only one card (Card 5) is the environment portrayed as benign and supportive, and this represents an idealised wish-fulfillment fantasy notable for its complete absence of conflict — suggesting that the patient's conception of a "good world" can only exist as an unrealistic, compensatory construction.

Figures Seen As:

Parental figures are perceived in a markedly split manner. Father figures are either violently dangerous (Card 1: cannibalistic murderer), emotionally blocked (Card 3: loves but cannot express), absent/negligent (Card 9: works out of station), blindly ineffective (Cards 6, 8: cannot see favouritism or abuse), or passively submissive to the point of destruction (Card 7: "dab ke rehta tha"). No card presents a father who is simultaneously present, protective, and emotionally available.

Mother figures demonstrate even more pronounced splitting. The biological mother is consistently idealised — she dies performing a nurturing act (Card 1: cooking), is devoted and misunderstood (Card 4), or is taken by blameless disease (Card 8: cancer). In contrast, replacement/step-mothers are demonised (Card 7: cannibalistic evil mother; Card 8: abusive step-mother "Lucy"). This Madonna-whore split in maternal representations suggests significant difficulty integrating ambivalent feelings toward the maternal object.

Sibling figures are perceived through the lens of parental favouritism. The younger sibling is consistently the "favourite" who receives unconditional love without merit (Cards 2, 6), while the older/middle sibling (the patient's identification figure) is the responsible, competent but emotionally abandoned caretaker.

Significant Conflicts:

The primary conflicts reflected across the protocol include: - Emotional need for parental love versus the experience of rejection and favouritism - Aggressive impulses versus their prohibition and consequent self-directed destruction (retroflexion) - Competence and responsibility versus lack of recognition or reward - Need for assertion versus fear of annihilation (Card 7: speaking up = death) - Dependence on dangerous/inadequate caregivers versus forced premature autonomy - Life instinct versus death instinct (Card 2: self-sacrifice as "logical") - Oral incorporation needs versus oral-aggressive anxieties (feeding/being eaten)

Nature of Anxieties:

The nature of anxieties reflected in the narratives includes annihilation anxiety (fear of being consumed, destroyed, or eliminated), abandonment anxiety (being left with unreliable substitutes, being emotionally invisible to parents), moral anxiety (guilt regarding aggressive wishes toward parents), and interpersonal anxiety (fear of being unvalued, expendable, and insignificant within the family hierarchy). Physical harm anxiety is prominent across 8 of 10 cards. Of particular clinical significance is the existential anxiety expressed in Card 2 — the patient questions the very purpose of his existence, concluding that his death would constitute the "least loss" to the family system.

Main Defenses:

The patient predominantly utilises primitive and neurotic defense mechanisms. Splitting is pervasive across the protocol — objects are either all-good or all-bad with no integration of ambivalence (biological mother = perfect; step-mother = evil; parents = either neglectful or idealised). Projection is the primary mechanism for managing aggressive impulses — massive aggression is attributed to parental figures (cannibalistic fathers and mothers) while the hero remains passive and victimised. Idealization is employed extensively for maternal figures (Card 4: monument built; Card 8: biological mother dies blameless) and desired relational states (Card 5: perfect twin bond).

Turning against the self (retroflexion) is a clinically significant defense observed in Card 2, where aggression toward the family for their favouritism is converted into self-sacrifice/self-destruction. Rationalization supports this defense ("mere marne se loss kam hai"). Denial and reaction formation are employed to manage anger toward parents — no direct expression of rage toward parental favouritism is permitted across the entire protocol.

Sublimation and counterphobic action appear only in Card 8, representing the patient's highest level of defensive functioning and suggesting that these mature defenses exist as latent capacities that are not habitually mobilised.

Adequacy of Superego:

The superego functioning across the protocol is inconsistent, externally dependent, and at times severely punitive. In Card 2, the superego is extremely harsh — the "crime" of merely existing without being exceptional or favoured is "punished" with death, suggesting massive superego pathology consistent with depressive dynamics. In Card 7, the superego is absent for the aggressor (evil mother escapes justice) but harshly punitive for the assertive victim (speaking up = death). In Cards 8 and 9, superego functioning is more proportionate and reality-based.

Across the protocol, punishment and justice are predominantly delivered by external institutional authorities (police) rather than through internalized moral regulation, suggesting that the superego functions primarily through fear of external consequences rather than internalized ethical standards.

Integration of the Ego:

The overall ego integration is variable but predominantly compromised. Formal thought processes remain intact across all cards — narratives are coherent, logically structured, and temporally sequenced. Reality testing is preserved — stories are internally consistent and resolutions are realistic within their narrative logic. These findings indicate adequate cognitive and intellectual resources.

However, the ego is significantly overwhelmed by aggressive drive content (cannibalistic themes breaking through in two cards), depressive cognitions (self as worthless, expendable), and primitive defensive operations (pervasive splitting, projection). The ego's synthetic function is compromised by the inability to integrate ambivalent feelings toward parental objects. Drive regulation is inadequate — aggressive and depressive content dominates most narratives without sufficient modulation.

Card 8 represents the patient's ego ideal and demonstrates that, when the narrative framework permits, the ego can mobilise adaptive coping strategies including perseverance, confrontation, sublimation, and delayed gratification. This represents significant therapeutic potential.

DIAGNOSTIC FORMULATION

The index patient, AP, a 16-year-old male adolescent, presents a CAT-H profile characterised by significant personality disturbance centred around themes of emotional deprivation, parental inadequacy, sibling-based worthlessness, and pervasive aggressive content with inadequate containment.

The psychological testing reveals a core constellation of:

1. **Depressive features with passive suicidal ideation** — as evidenced by Card 2's explicit narrative of self-sacrifice as "logical" for someone whose death would cause the "least loss," parental indifference to the hero's death, and pervasive themes of worthlessness and emotional invisibility across the protocol.
2. **Severe object relations disturbance** — characterised by pervasive splitting (all-good/all-bad representations), inability to integrate ambivalent feelings toward parental figures, absence of a "good-enough" parental object across the protocol, and pre-oedipal level functioning in interpersonal representation.
3. **Aggressive drives with inadequate ego containment** — manifested through oral-cannibalistic content in two cards (1, 7), extreme projected violence across 8 of 10 cards, and repetitive themes of annihilation and devouring. The aggressive content is predominantly managed through projection and splitting rather than sublimation or neutralisation.
4. **Attachment insecurity consistent with disorganised attachment** — no card presents a secure base; caregivers are consistently unreliable, dangerous, absent, or emotionally depriving; the patient demonstrates forced self-reliance ("hume khud ke liye hi karna hai") as a premature coping strategy born from repeated experiences of parental failure.
5. **Parentification and role reversal** — the patient consistently identifies with the responsible child who must parent younger siblings (Card 6), protect the mother's reputation (Card 4), or inform the father of reality (Card 8), without receiving reciprocal care or recognition.
6. **Core sibling rivalry wound** — the most emotionally charged content centres on perceived parental favouritism toward a younger sibling, with the patient experiencing himself as the "invisible middle child" whose competence and responsibility earn no love.
7. **Inhibition of self-expression and assertion** — Card 7 explicitly equates speaking up against authority with death, and across the protocol, the patient's heroes consistently suppress their feelings ("hum theek hai"), accept unfair treatment without protest, and direct anger inward rather than outward.

The patient's ego, while cognitively intact with preserved reality testing and adequate formal thought processes, is overwhelmed by primitive aggressive and depressive

content and relies heavily on immature and primitive defense mechanisms (splitting, projection, idealization, denial). Mature defensive operations (sublimation, counterphobic action) exist as latent capacities demonstrated in Card 8, indicating therapeutic potential.

The overall profile suggests significant internal conflict, emotional vulnerability, depressive orientation with passive suicidal ideation, and impaired interpersonal functioning rooted in perceived familial rejection. The patient appears psychologically conflicted and emotionally distressed but not disorganised, with preserved reality orientation. Structured therapeutic intervention focusing on self-worth enhancement, emotional expression, anger management, family dynamics exploration, and strengthening of adaptive defenses would be indicated.

CLINICAL RECOMMENDATIONS

1. **Immediate:** Formal suicidal ideation assessment warranted based on Card 2 content (passive suicidal themes framed as "logical" self-sacrifice).
 2. **Family assessment:** Exploration of actual family dynamics regarding parental favouritism, sibling differential treatment, and emotional availability.
 3. **Individual therapy:** Focus on self-worth, emotional expression, anger acknowledgment, and strengthening mature defenses (sublimation, assertiveness).
 4. **Domestic environment screening:** Card 7's domestic violence narrative warrants exploration of witnessed or experienced violence.
 5. **Attachment-focused work:** Addressing early attachment disruption and building capacity for secure relational functioning.
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Report prepared using Bellak's 10 Variables System for CAT-H Interpretation (Leopold Bellak, M.D., Fifth Edition) with Murray's Need-Press Framework for drive analysis.